

# Proposed Primary Care Dementia Diagnostic Pathway

## Introduction

With the national drive to increase dementia diagnosis rates in order to allow more people to access dementia support, it has been recognised for some time that diagnosis will sometimes need to occur outside the traditional memory clinic model. Having diagnosis occurring only in memory clinics reduces equity as some people are unable to travel, particularly the frail and elderly.

In recognition of this a number of primary care diagnostic pathways have been introduced in individual Clinical Commissioning Groups (CCGs) and have now been operating successfully for several years. The South Gloucestershire model was (with Bristol) the first to be implemented in 2012, and has now been adopted by the South West Clinical Network (SWCN) as its recommended model. However, there is no nationally accepted model for primary care diagnosis. NICE in its 2018 review of dementia services stated that “A referral to a diagnostic service does not have to involve a clinic appointment. People can be seen in community settings (such as their home or a care home), or advice can be provided to the referrer without a formal appointment being made”<sup>1</sup>.

In light of this, it feels an appropriate time to try to provide a NICE-approved primary care pathway (PCP) for dementia diagnosis. In doing this I have drawn on my experience of setting up and operating the South Gloucestershire PCP, presentations we have had to the SWCN of different pathways, as well as a review of wider national and international pathways<sup>2,3,4,5,6</sup>. I recognise, as the NICE update does, that there is not a single ideal model: the best local solution will depend on the way dementia services are managed locally<sup>7,8</sup>. In addition, there is debate amongst specialists over some areas, for instance the value of imaging in the frail elderly. However, evaluations of such PCPs have shown them to be safe, accurate and well-received by patients.<sup>9</sup>

The current local guidelines share a number of common features, so after discussion with colleagues I have amalgamated them into a pathway suitable for use by GP’s who wish to diagnose dementia in primary care as envisaged in NG 97 and QS 184. The details can be varied according to local services (e.g. exclusion criteria, availability of open access imaging, access to dementia specialists).

The proposed pathway is set out on page 2, with supporting information and examples of existing pathways appended for comparison.

P J Bagshaw September 2019

## Primary Care Diagnostic Pathway

### Memory loss suspected

1

Take a history: Has there been a gradual deterioration in **memory** and **cognitive function**? (if NO, dementia is unlikely). Seek corroboration from carer if possible. Conduct a physical examination

Is this **sufficiently severe** to impact on activities of daily living (if NO, refer to memory clinic to exclude mild cognitive impairment)

2

Carry out **Mini-Cog** or similar

If **borderline** or **no** memory problem disclosed, patient may still have mild dementia, refer to memory clinic for in-depth testing

If **positive** (fails), dementia is likely. If fits agreed local criteria (e.g. too frail to travel, over 75) proceed to testing. Otherwise, or if any doubt, refer to memory clinic

3

**Blood tests:** FBC, CRP, C&Es, folate, HbA1c, LFTs, B12, TFTs and Calcium. Consider urine dip/MSU and CXR if infection suspected

**Imaging:** CT head unless refuses/too frail. If >80 years, only scan if head injury, suspected malignancy or neurological signs/symptoms

4

**Exclude other diagnoses** e.g. delirium (usually rapid onset), depression

**Take a drug history:** stop opiates/anticholinergics/sedatives unless essential

5

If a GP with **appropriate specialist knowledge**, a diagnosis of dementia can be made; otherwise, consult with local dementia specialist as per local protocol

Donepezil or other **AChEIs** may be of some value if Alzheimer's or mixed (see attached protocol if primary care prescribing agreed locally).

Address vascular risks, particularly reversing metabolic syndrome and keeping active

Refer to **local post-diagnostic support network** (Dementia advisor, Alzheimer's society or similar)

Agree a care plan. Arrange follow-up for 6 weeks if starting AChEI, or 3-6 months

## Notes to the proposed pathway:

**Take a history.... Seek corroboration from carer if possible** “At the initial assessment take a history (including cognitive, behavioural and psychological symptoms, and the impact symptoms have on their daily life) from the person with suspected dementia and if possible, from someone who knows the person well (such as a family member).” (NICE NG 97)

**Conduct a physical examination** NG 97 1.2.2

**Is this sufficiently severe to impact on activities of daily living (if NO, refer to memory clinic to exclude mild cognitive impairment)** See Rasmussen Kipps and Bagshaw Management of Mild Cognitive Impairment.

**Carry out Mini-Cog or similar.** “When using cognitive testing, use a validated brief structured cognitive instrument such as: the 10-point cognitive screener, the 6-item cognitive impairment test, the 6-item screener, the Memory Impairment Screen, the Mini-Cog, Test Your Memory”. (NICE NG 97). Such tests will not rule out mild dementia (see NG 97 1.2.4) but if failed point to a significant defect. In the future, machine learning algorithms such as DECODE may improve accuracy further.

**Blood tests.** <https://www.nhs.uk/conditions/dementia/diagnosis-tests/> FBC and calcium are additional tests in most GP “dementia screens”, and CRP or other inflammatory markers are often recommended to exclude infection. If patients lack consent under the MCA act a best interests decision should be made.

**Imaging.** Perhaps the most debated subject: the critical thing is whether imaging will change management. NG 97 gives excellent advice on imaging in memory clinics but other authors (e.g. Harper, L. An algorithmic approach to structural imaging in dementia *Journal of Neurology, Neurosurgery and Psychiatry* Vol 85 2013) discuss the most appropriate imaging in other circumstances. As most dementia is mixed in the elderly, sub-typing is of less importance in this age group. Excluding tumours and other treatable pathologies remains important except in the very frail where intervention would be inappropriate: clinical decision-making is important, and local guidelines will be critical.

**Exclude other diagnoses** <https://bestpractice.bmj.com/topics/en-us/242> gives a primary care-focussed account of differential diagnosis, with delirium, depression and mild cognitive impairment cited as the main diagnoses to consider. If sub-typing of dementia is required, this should be done via a memory clinic, although imaging can give clues.

**Take a drug history** Anticholinergics, sedatives and opiates both increase the risk of dementia, and worsen cognitive function in those with memory problems, so stopping them is high priority, see for instance <https://www.nhs.uk/news/medication/widely-used-class-drugs-linked-dementia/>

**If a GP with appropriate specialist knowledge, a diagnosis of dementia can be made; otherwise, consult with local dementia specialist as per local protocol** “Specialists are those with the appropriate knowledge and skills, and include secondary care medical specialists (for example, psychiatrists, geriatricians and neurologists) and other healthcare professionals (for example, GPs, nurse consultants and advanced nurse practitioners) with specialist expertise in assessing and diagnosing dementia”. (NICE QS 184) This varies widely from area to area, so generic advice only can be given: the area-specific advice should be inserted as appropriate.

**Donepezil or other AChEIs may be of some value if Alzheimer’s or mixed (see attached protocol if primary care prescribing agreed locally).** Although generally Amber (specialist-initiated) drugs,

primary care protocols for GP initiation have been used successfully for several years. The reference to this can be amended depending on local guidelines.

**Address vascular risks, particularly reversing metabolic syndrome and keeping active** Although more important in prevention (see for instance <https://pjbagshaw.wordpress.com/dementia/> ) there is early evidence that improving metabolic health may slow the progression of dementia.

**Refer to local post-diagnostic support network** Again, this will vary enormously from area to area, and this generic advice should have CCG-specific information inserted

From experience, we have found it useful to have pathways available on a single A4 sheet which can be kept in a GP surgery (and will be legible if produced in monochrome). The proposed pathway has therefore been deliberately slimmed down to the minimum: a layer of additional information and caveats could be added which would make it unwieldy. Local areas who adopt this plan should insert variations as appropriate; this particularly applies to cognitive testing tools, imaging, available specialists and post diagnostic support. The above is therefore an assured proforma for local areas who wish to adopt primary care diagnosis to use as a starting point, adapting it to local needs and resources. I have found that many areas wish to adopt such a scheme to ensure timely diagnosis and equity for their frail elderly population, but are nervous of doing so, often owing to a misunderstanding of the NICE meaning of “specialist”. With the welcome clarification contained in QS 184, and the collective experience of existing pathways running for several years, several CCGs have expressed a wish to have a NICE-approved pathway to reassure GPs and others in primary care.

## References

1. NICE Dementia assessment and diagnosis June 2018 update; Dementia Quality standard [QS184] June 2019 <https://pathways.nice.org.uk/pathways/dementia>
2. De Lepeleire, J. et al, 'The primary care diagnosis of dementia in Europe: An analysis using multidisciplinary, multinational expert groups', *Aging & Mental Health*, 12:5, 568 — 576
3. Bagshaw, P.J. et al “Proposal for memory service pathway”, South Gloucestershire CCG, Jun 2012
4. Gibbs, J et al “Guidelines on diagnosing Alzheimer’s disease in Primary Care” BNSSG CCG 2016
5. Anderson, J. et al “Primary Care Memory Assessment Service” Oxfordshire CCG 2017
6. DiADeM [www.yhscn.nhs.uk/media/PDFs/mhdm/Dementia](http://www.yhscn.nhs.uk/media/PDFs/mhdm/Dementia)
7. Greening L, Greaves I, Greaves N, Jolley D: Positive thinking on dementia in primary care: Gnosall Memory Clinic. *Community Practice*. 2009, 82 (5): 20-23.
8. Banerjee S, Willis R, Matthews D, Contell F, Chan J, Murray J: Improving the quality of care for mild to moderate dementia: an evaluation of the Croydon Memory Service Model. *Int J Geriatr Psychiatry*. 2007, 22: 782-788. 10.1002/gps.1741.
9. Dodd, E. et al, “An evaluation of primary care led dementia diagnostic services in Bristol” BMC Health Services Research Volume 14, Article number: 592 (2014)

## Appendices

### 1 DiADeM diagnostic tool

| DiADeM Tool                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
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| Diagnosing Advanced Dementia Mandate (for care home setting)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| A diagnosis of dementia can be made with a high degree of certainty if all five criteria listed below are met:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <b>1</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <b>Functional impairment</b><br>The person is no longer fully independent in relation to basic activities of daily living, washing, dressing, feeding and attending to own continence needs. The requirement of prompting or supervision of staff constitutes a loss of full independence.                                                                                                                                                                                                                                        | <input style="width: 40px; height: 30px;" type="checkbox"/> |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <b>2</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <b>GPCOG</b><br>NB Assessment tools other than GPCOG can be used                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <table border="1" style="width: 100%; border-collapse: collapse;"> <thead> <tr style="background-color: #0070C0; color: white;"> <th style="width: 70%; padding: 5px;">Step 1: Patient Assessment</th> <th style="width: 30%; padding: 5px;">Score achieved</th> </tr> </thead> <tbody> <tr> <td style="padding: 5px;">The total score for all six questions (scored out of 9)</td> <td style="text-align: center; height: 20px;"></td> </tr> <tr> <td colspan="2" style="padding: 5px;"> <ul style="list-style-type: none"> <li>- Patient scores 9/9 = no cognitive impairment, no need to proceed to carer interview</li> <li>- Patient scores 5-8 = possible cognitive impairment, proceed to carer interview</li> <li>- Patient scores 0-4 = cognitive impairment is likely; no need to proceed to carer interview. Conduct standard investigation</li> </ul> </td> </tr> <tr style="background-color: #0070C0; color: white;"> <th style="padding: 5px;">Step 2: Carer Interview (Someone who knows the patient well)</th> <th style="padding: 5px;">Score achieved</th> </tr> <tr> <td style="padding: 5px;">The total score for all six questions scores out of 6 to get a total score, only add the number of items answered 'no', don't know or Not Applicable</td> <td style="text-align: center; height: 20px;"></td> </tr> <tr> <td colspan="2" style="padding: 5px;">                     Carer score 0-3 = cognitive impairment is likely. Conduct standard investigations.<br/>                     Carer score 4-6 = refer to the Memory Support Service for re-screening in 6 months                 </td> </tr> </tbody> </table> |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             | Step 1: Patient Assessment | Score achieved | The total score for all six questions (scored out of 9) |  | <ul style="list-style-type: none"> <li>- Patient scores 9/9 = no cognitive impairment, no need to proceed to carer interview</li> <li>- Patient scores 5-8 = possible cognitive impairment, proceed to carer interview</li> <li>- Patient scores 0-4 = cognitive impairment is likely; no need to proceed to carer interview. Conduct standard investigation</li> </ul> |  | Step 2: Carer Interview (Someone who knows the patient well) | Score achieved | The total score for all six questions scores out of 6 to get a total score, only add the number of items answered 'no', don't know or Not Applicable |  | Carer score 0-3 = cognitive impairment is likely. Conduct standard investigations.<br>Carer score 4-6 = refer to the Memory Support Service for re-screening in 6 months |  |
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| Cognitive impairment indicated on memory screening                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | <input style="width: 40px; height: 30px;" type="checkbox"/> |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <b>3</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <b>Corroborating History</b><br>History of gradual cognitive decline (typically for the last few years) is confirmed by care staff, relatives and medical records. Staff/relatives confirm that in their opinion the patient consistently demonstrates both functional and cognitive impairment.                                                                                                                                                                                                                                  | <input style="width: 40px; height: 30px;" type="checkbox"/> |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <b>4</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <b>Investigations</b><br>Dementia screening <b>bloods are normal</b> (where clinically appropriate and patient consents to bloods). If patient lacks capacity to consent to bloods, a best interest decision must be made and documented accordingly. NB. If intracranial pathology (e.g. subdural haematoma, cerebral tumour) is suspected, referral for a brain scan may be appropriate. Otherwise where dementia is advanced, differential diagnosis is unlikely to affect patient management and a brain scan is unnecessary. | <input style="width: 40px; height: 30px;" type="checkbox"/> |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <b>5</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | <b>Exclusion Criteria</b><br>There is <b>no acute underlying cause to explain</b> confusion i.e. delirium (acute confusional state) has been excluded. Mood disorder or psychosis is also excluded.                                                                                                                                                                                                                                                                                                                               | <input style="width: 40px; height: 30px;" type="checkbox"/> |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <p>A diagnosis of dementia can be made with a high degree of certainty if <b>all five</b> criteria listed above are met. If dementia is confirmed, please add this patient to your GP practice dementia register using the recommended <a href="#">codes</a>. Consent should be sought for this from the person themselves or a family carer where the individual lacks capacity.</p> <p><b>NB. Where a diagnosis of dementia is confirmed, a copy of the completed DiADeM tool should be saved into the patient's notes as it forms part of their clinical record.</b></p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <p><sup>1</sup> "Guidance for Commissioners of Dementia Services", published by The Joint Commissioning Panel for Mental Health states patients who present with advanced symptoms of dementia can be diagnosed and managed by primary care with or without CMHT help. <a href="http://www.knosh.info">www.knosh.info</a>. Thanks to: Dr Graeme Finlayson, Bradford District Care NHS FT and Dr Subha Thiyagesh, South West Yorkshire Partnership NHS FT.</p> <p>© Yorkshire and Humber Strategic Clinical Network and Code4Health. This publication is licensed under the Open Government Licence v3.0 except where otherwise stated. To view this licence, visit <a href="http://www.nationalarchives.gov.uk/doc/open-government-licence/version/3">http://www.nationalarchives.gov.uk/doc/open-government-licence/version/3</a> or write to the Information Policy Team, The National Archives, Kew, London TW9 4DU. Any enquiries regarding this publication should be sent to: <a href="mailto:ENGLAND.YHSCN@nhs.net">ENGLAND.YHSCN@nhs.net</a></p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |
| <p><b>Disclaimer:</b> Healthcare professionals must make their own decisions about assessment and care on a case-by-case basis, using their clinical judgement, knowledge and expertise and in consultation with other key staff and family carers. This tool is not intended to replace physician judgment in assessing individual patients. Ratification of this tool for local use should follow the usual process within all affected organisation(s). Departure from local prescriptive protocols or guidelines should be fully documented in the patient's case notes at the time the relevant decision is taken. The authors of this tool accept no responsibility for any inaccuracies or information perceived as misleading. The authors assume no legal liability or responsibility for the accuracy, completeness or clinical efficacy of this guidance.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                                                             |                            |                |                                                         |  |                                                                                                                                                                                                                                                                                                                                                                         |  |                                                              |                |                                                                                                                                                      |  |                                                                                                                                                                          |  |

## 2 Gloucestershire CCG Primary Care Medical Assessment Service

### PCMAS – the 3 stage pathway

- ❑ 1<sup>st</sup> stage: referral to PCMAS (routine GP consultation), GPCOG, medication screen
- ❑ 2<sup>nd</sup> stage: 20 min practice nurse appointment for routine screening and information gathering (PHQ9, Bristol Activities of Daily Living, repeat GPCOG + IQ, blood and urine screen, hearing/eyesight, carers)
- ❑ 3<sup>rd</sup> stage: 20 min GP appointment to review data, clarify history and make diagnosis of either mild memory disturbance or dementia. CT scan arranged at this point if required and a trial of AChEi considered. Dementia care plan
- ❑ Follow up arranged as appropriate

### Recommendation for requesting CT scans in PCMAS

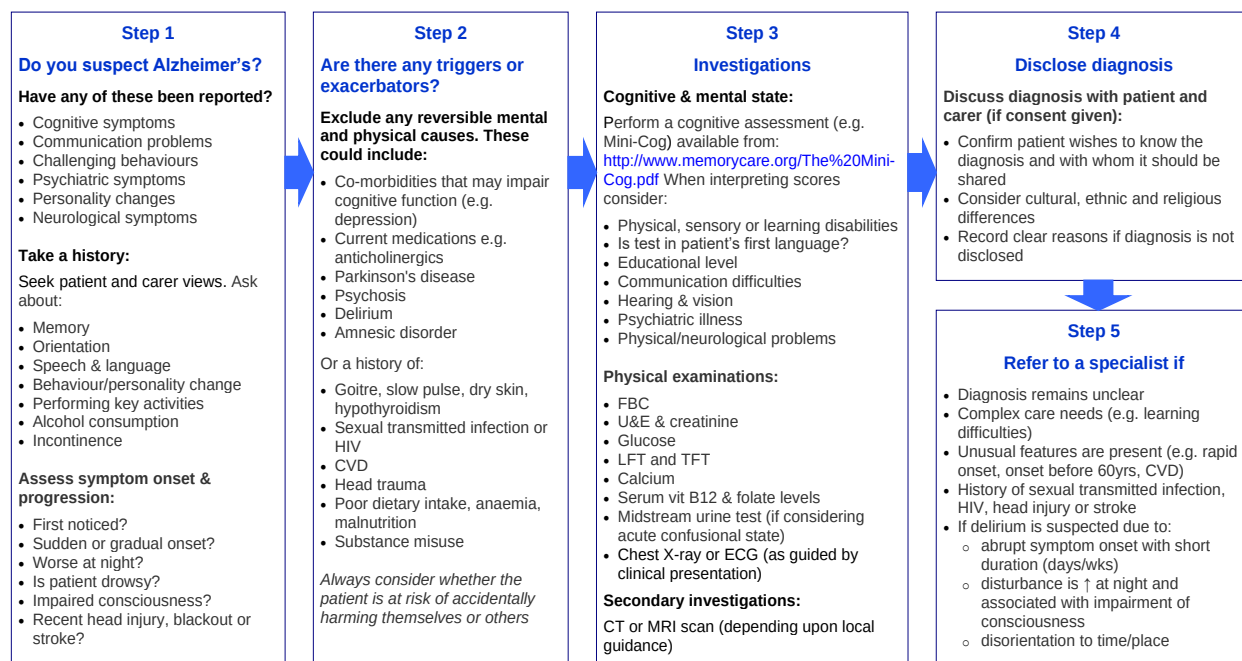
| Age 65 or less                                                        | Age 65-79                                                                                | Age 80+                                                                                                                                                                                                                                                                     |
|-----------------------------------------------------------------------|------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Not suitable for primary care management-refer secondary care service | Arrange scan unless history of gradual memory decline over 1 yr and no atypical features | Only scan if: <ul style="list-style-type: none"> <li>• Recent head injury</li> <li>• History of malignancy</li> <li>• Rapid unexplained deterioration</li> </ul> Unexplained focal neurological symptoms and signs including early urinary incontinence or gait disturbance |

## 3 Bristol, North Somerset and South Gloucestershire CCG Primary Care Guidelines

### Guidelines on diagnosing Alzheimer's disease in Primary Care

#### Diagnosis

Diagnosis of Alzheimer's disease (AD) is primarily based on clinical features, after excluding other systemic and brain disorders that could account for cognitive impairment. Onset is typically insidious and progresses slowly over 7-10 years. In the early stages of AD, people commonly have memory lapses and experience problems with executive function (a set of cognitive abilities that control and regulate other abilities and behaviours). As the disease progresses, other symptoms become apparent e.g. difficulties with language, planning and decision making.



### Diagnosing Dementia in Primary care

Step 1

#### Memory Problem Suspected

From patient, hospital, screening, other alerts from primary care

Use **mini-cog** to screen for memory problems

If negative, reassure or review 6 months at GP discretion (for instance if patient/family feel deterioration in mental functioning but insufficient to score

Step 2

#### If memory problems confirmed

Review **medication**

Exclude **depression, infection**

**Blood tests:** FBC, C&Es, folate, glucose, LFTs, B12, TFTs and Ca.

**CT** (unless inappropriate) Head views with Coronals: ask “is there sufficient neurological evidence to support a diagnosis of vascular dementia?”

Seek information from **carers** where possible

Step 3

**Discuss with memory nurse** with bloods and CT results if appropriate. (If confident, GP can make diagnosis and initiate treatment if uncomplicated, patient over 75) **Memory nurse** will advise. Depending on patient, may carry out further testing, agree diagnosis or refer to memory clinic

Step 4

At diagnosis, **see patient**

**Give diagnosis** (this may sometimes be mixed or suspected)

**Give information and support** to patient and carers (see support pack)

Step 5

If **vascular**, address vascular risks.

If **Alzheimer's** or mixed, consider prescribing Donepezil (see guidelines)

Step 6

**Follow up** at 3-6 months or as Donepezil guidelines

## Guidelines on Initiating and Prescribing Donepezil in Primary Care



Bristol Primary Care Trust  
North Somerset Primary Care Trust  
South Gloucestershire Primary Care Trust

### Key Points:

- Donepezil is used for the treatment of mild to moderate Alzheimer's disease (AD) and in mixed vascular dementias where there is a significant AD component
- Donepezil is the first-line acetylcholinesterase inhibitor (AChEi) of choice in BNSSG primary care
- Do **not** prescribe donepezil if the patient is already taking another AChEi
- Treatment should **only** be started if a care giver is available to regularly monitor medication compliance and its effect
- Side effects are possible for up to 20% of patients and are greatest on starting therapy or at dose increase, but usually diminish in a few days
- Patients and carers should be advised not to expect a dramatic reversal of cognitive decline. Expected benefits include improved functioning and behaviour
- Treatment should **only** be continued when it is considered to be having a worthwhile effect on cognitive, global, functional or behavioural symptoms

Check

1

- **Confirm diagnosis of AD** (see separate guidelines on diagnosing Alzheimer's)
- **Use the Mini-Cog test to establish current cognitive impairment** (if not done already) - <http://www.memorycare.org/The%20Mini-Cog.pdf>
- **Consider carer/relatives views** before initiating treatment (if patient consents)
- **Consider co-morbidities** (Table 1)
- **Review current medication and consider potential drug interactions** (Table 2)
- Involve the patient in decisions and check they understand and have not been coerced into agreeing to treatment

Prescribe

2

- **Prescribe: donepezil tablets 5mg nocte for 4 weeks.** Use orodispersible tablets if the patient has swallowing difficulties or severe behavioural & psychological symptoms
- **Review patient after 4 weeks.** Increase dose to 10mg nocte if necessary (Note: many patients will be adequately maintained on 5mg). **Maximum daily dose is 10mg**
- In renal/hepatic impairment (closely monitor for adverse effects):  
  
Renal impairment: eGFR 30-89ml/min: no dose adjustment required. <29ml/min: titrate normal dose with caution according to response.  
  
Hepatic impairment: Mild to Moderate: perform dose escalation according to individual tolerability. Severe: contraindicated

Review

3

- **Review treatment effect at 3 months** using the same cognitive assessment test
- Consider physical, sensory or learning disabilities and communication difficulties that could affect the result
- **Consider carer's opinion** on any improvement - the IQCODE informant questionnaire can be useful in evaluating this and is available from [http://www.patient.co.uk/doctor/Informant-Questionnaire-on-Cognitive-Decline-in-the-Elderly-\(IQCODE\).htm](http://www.patient.co.uk/doctor/Informant-Questionnaire-on-Cognitive-Decline-in-the-Elderly-(IQCODE).htm) (Note: Carers often report improvements which are not picked up by scales)
- **Check patient tolerance and the presence of side effects** (Table 3). If side effects are intolerable, refer to a specialist
- **Treatment should be continued if it is having a worthwhile effect** on cognitive, global, functional or behavioural symptoms. If the patient is not responding to treatment, refer to a memory specialist
- Patients who are stable on donepezil should be **reviewed every 3-6 months**